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NHS North Central London Integrated Care Board

Acute Dizziness in Adults Primary Care Clinical Pathway

For any queries regarding the content of this document e-mail: nclicb.pathways@nhs.net

Key

- Pi — "Must do" actions for GP's / (Triaged by RSS)
- B — Recommendations for Primary Care
- R — Red flag / urgent referral
- O — Routine referral
- P — Public health intervention
- G — Audio-visual aids for patients and GP
- Click icon for clinical evidence
- Right-click to use hyperlink

Note: Alpha-numeric step references are to aid printing in black & white and colour blindness

1.0 (B)

Assessment

- **History:**
  - Vertigo: Illusion of motion/spinning (vestibular network)
  - Syncope: Light headedness, faint or giddy (usually cardiovascular)
  - Review medication/drugs/alcohol.
  - Eye witness account
- **Examination:**
  - Heart rate and rhythm; Blood pressure (including postural blood pressure); Neurological examination (especially cranial nerves. Look for nystagmus, cerebellar signs, gait); Otoscopy; Dix-Hallpike to assess for benign paroxysmal positional vertigo (BPPV) (see Dix-Hallpike video[1] on GP website)

DVLA states that people with liability to sudden and unprovoked or unprecipitated episodes of disabling dizziness should stop driving and inform the DVLA. See DVLA guidance - Neurological disorders: assessing fitness to drive

- **ECG and blood tests** (FBC, U&E, TFT, HbA1c, Vitamin B12) prior to referral

1.1 (Pi)

2.0 (R)

Red flag symptoms

No

Yes

3.0 (B) **BPPV**

Diagnosis
Brief positional spinning vertigo. Dizziness turning over in bed. Rotatory nystagmus on Dix-Hallpike

12.0 (B)

Management
Offer Epley [2] or Semont [3] manoeuvre **if trained**. Be cautious if the person has neck or back problems, or cardiovascular problems such as carotid sinus sensitivity.

Seek A&G if uncertain

4.0 (B) **Vestibular Neuritis**

Diagnosis
Continuous un-provoked vertigo, nausea, and unsteadiness. No other symptoms.

13.0 (Pi)

Management
Symptoms usually self-limiting, and resolve within a few weeks. Offer treatment if symptoms severe Cinnarizine 30mg TDS OR Prochlorperazine 5mg TDS up to three days if required** Encourage mobility

5.0 (B) **Vestibular Migraine**

Diagnosis
Recurrent episodes of vertigo (spinning or rocking sensations), nausea, motion intolerance, photophobia, phonophobia. Normal exam

14.0 (Pi)

Management
Cinnarizine 30mg TDS OR Prochlorperazine 5mg TDS up to three days if required.

For other migraine treatments (See NCL Adult Headache Pathway)
(add link to Headache)

6.0 (B) **Syncope**

10.0 (B)

See Loss of Consciousness pathway
(add link to TLOC pathway)

11.0 (R)

Vertigo associated with any of the following:

- Cranial nerve, limb motor/sensory symptoms or ataxia **OR**
- Acute onset unilateral hearing loss
- Acute onset headache

15.0 (R)

- If acute unilateral hearing loss is the **predominant symptom**
- Urgent referral to ENT on-call at the local hospital (via switchboard)

16.0 (R)

- Very recent onset (secs to hours)
- If post-trauma/ fall or any FAST sympto ms (Facial drooping, Arm weakness, Speech difficulties and Time)
- Call 999 for emergency assessment.

17.0 (R)

- Recent onset (<1 week)
- Emergency hospital admission where clinically appropriate OR Urgent 2WW referral to the Suspected Brain and CNS cancer pathway if the patient meets the criteria [4]

18.0 (O)

If not settled after two weeks, consider referral to ENT community clinic, secondary care clinic (audiovestibular medicine, ENT or neurology) or local care of the elderly / falls clinic as appropriate

General Principles

- NCL Training hub acute vertigo video training
- **AVOID use of pharmacotherapy for more than 3 days in dizziness or vertigo as there is little evidence of benefit and prolonged use may delay natural central compensation and create psychological dependence. Anti-emetic medication can be used regularly for three days, and then more sparingly for a further week, gradually reducing the dose over this time, if needed. During this time, patients should be encouraged to move and begin more normal activities as this helps with the compensation process.
- All patients should receive general supportive and reassurance advice as a significant proportion develop secondary avoidance behaviour. They should be advised to mobilise as much as possible as this helps them compensate quicker.
- NOTE: Consider Advice & Guidance on e-referral to ENT or Neurology.
- DVLA states that people with liability to sudden and unprovoked or unprecipitated episodes of disabling dizziness should stop driving and inform the DVLA. See DVLA guidance - Neurological disorders: assessing fitness to drive
- Workplace —Advise person to inform their employer if their dizziness poses a risk in the workplace (for example if they use ladders, operate heavy machinery, or drive a vehicle).

18.1 (B)

This pathway has been developed from published guidance [5],[6] in collaboration with local neurologists, ENT and audiovestibular medicine specialists. The guidance is to assist GPs with clinical decision making and is not intended to replace clinical judgement

[1] Epley and Beyond: An Update on Treating Positional Vertigo (Dix-Hallpike). https://gps.northcentrallondon.icb.nhs.uk/education/video/epley-and-beyond-an-update-on-treating-positional-vertigo-dix-hallpike*

[2] Epley and Beyond: An Update on Treating Positional Vertigo (Epley). https://gps.northcentrallondon.icb.nhs.uk/education/video/epley-and-beyond-an-update-on-treating-positional-vertigo *

[3] Epley and Beyond: An Update on Treating Positional Vertigo (Semont). https://gps.northcentrallondon.icb.nhs.uk/education/video/epley-and-beyond-an-update-on-treating-positional-vertigo-semont *

[4] Suspected Cancer Two-Week Referrals (2WW): https://gps.northcentrallondon.icb.nhs.uk/service/suspected-cancer-two-week-referrals

[5] NICE Clinical Knowledge Summary - Vertigo. Last revised in November 2020. https://cks.nice.org.uk/topics/vertigo/

[6] NICE Clinical Knowledge Summary – Benign paroxysmal positional vertigo. Last revised in April 2022. https://cks.nice.org.uk/topics/benign-paroxysmal-positional-vertigo/

*Videos obtained from the original article - Kaski D, Bronstein AM. Epley and beyond: an update on treating positional vertigo. Practical Neurology 2014;14:210-221. BMJ Publishing Group Limited. Available via https://pn.bmj.com/content/14/4/210.info